

DOCTOR:

DATE OF SERVICE:

Pathology Consultation Report

Final

CASE:

ANATOMIC PATHOLOGY DIAGNOSIS:

- A - Ureter, right, distal, segmental resection
 - Focal low grade dysplasia
- B - Ureter, left, distal, resection
 - No pathologic diagnosis
- C - Apical urethral surgical margin, resection
 - Urothelial with reactive atypia
- D - Urinary bladder, radical cystectomy
 - Urothelial carcinoma of the urinary bladder (transitional carcinoma)
 - High grade (grade 3/3)
 - Solid pattern
 - 7 cm
 - Invasion into the inner half of the muscular propria
 - Multifocal lymphovascular invasion
 - All surgical margins (right and left ureteral, urethral and soft tissue margins)
 - free of tumor involvement
 - Carcinoma in situ/high dysplasia, multifocal, involving urothelium
 - Brunner's nest with high grade dysplasia/carcinoma in situ
 - Granulomatous inflammation of the mucosa
 - Reactive changes of the mucosa
 - Right ureter, high grade dysplasia/carcinoma in situ
 - Left ureter, high grade dysplasia/carcinoma in situ
 - Prostate, pending microscopic examination

- E - Left pelvic lymph nodes, dissection
 - One out ten lymph nodes with microscopic metastatic carcinoma
- F - Right pelvic lymph nodes, dissection
 - Two out of eleven lymph nodes with metastatic carcinoma
- G - Left common iliac lymph nodes, excision
 - Eleven lymph nodes without carcinoma
- H - Right common iliac lymph nodes, dissection
 - Six lymph nodes without metastatic carcinoma
- I - Presacral lymph nodes, resection

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TCGA-FJ-A3Z7-01A-PR

Redacted



1CD-0-3

Carcinoma, urothelial, NOS 8120/3

Site: bladder, NOS C67.9

hw
9/28/12

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History		☒
Local/Synchronous Primary Notes		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials:	7/28/12	
Date Reviewed:	7/28/12	

Six lymph nodes without metastatic carcinoma

J - Para-aortic lymph nodes, excision
Two lymph nodes without metastatic carcinoma

K - Precaval lymph node, excision
One lymph node without metastatic carcinoma

L - Discarded segment of bowel, segmental resection
Small bowel wall without diagnostic alteration

M - Proximal margin, left ureter, excision
No pathologic diagnosis

N - Distal right ureter, segmental resection
Focal low grade dysplasia

PATHOLOGIST:

I have reviewed this material and confirm the report. Released by
electronic signature on:

MATERIAL:

A. Right distal ureter; B. Left distal ureter; C. Apical margin. D.
Urinary bladder
E. Left pelvic lymph nodes. F. Right pelvic lymph nodes. G. Left
common iliac lymph nodes. H. Right common iliac lymph nodes. I.
Pre-sacral lymph nodes. J. Para-aortic lymph nodes. K. Pre-caval
lymph nodes. L. Discarded segment of small bowel.
M. Proximal margin of left ureter. N. Distal right ureter. O.
Prostate.

INTRAOPERATIVE DIAGNOSIS:

FROZEN SECTION DIAGNOSIS

AFS Right distal ureter excision
- Lower grade dysplasia versus high grade dysplasia; suggest
additional resection
- (Reported to

BFS Left distal ureter excision

- No dysplasia
NAME [REDACTED]

- No carcinoma
- (Reported to

CFS Atypical urethral margin, biopsy
- No dysplasia
- No carcinoma
(Reported to

HISTORY:

with bladder cancer.

GROSS:

A-Received fresh labeled as right distal ureter is a tan pink unoriented segment of ureter (0.2 x 0.2 x 0.2 cm). Entirely submitted for frozen section in AFS.

B-Received fresh labeled as left distal ureter is a tan pink unoriented segment of ureter (0.3 x 0.3 x 0.2 cm). Entirely submitted for frozen section in BFS.

C-Received fresh labeled apical ureteral margin is a tan-pink, unoriented, irregular piece of tissue measuring 1x0.5x0.3cm. Entirely submitted for frozen section in CFS.

D. A radical cystoprostatectomy specimen consisting of a urinary bladder (9.0 x 7.0 x 2.0 cm when opened); a prostate gland; and attached perivesical adipose tissue (overall measurements: 15.0 x 6.0 x 4.0 cm).

Located in the anterior, posterior, right, and left walls of the urinary bladder, there is a nodular firm lesion (7.0 x 4.5 cm). Sections through the lesion exhibit a firm white lesion that extends to a depth of 2.0cm into the muscularis and adipose tissue. The ureters are not identified in the specimen. The remainder of the urinary bladder mucosa is edematous and red. The specimen is sequentially sectioned inferior portion to the dome, and representative sections are submitted in thirty-five cassettes: D1-D35.

Ink code: Blue - anterior
Black - posterior
Red - right
Orange - left

Section code:

D1 Anterior right bladder wall
D2 Posterior bladder wall
D3 Posterior bladder wall
D4 Anterior left bladder wall
D5 Anterior right bladder wall
D6 Right bladder wall

D7 Left bladder wall
D8 Right anterior bladder wall

D9 Right posterior bladder wall
D10 Left posterior bladder wall
D11 Right posterior bladder wall (possible ureter)
D12 Left posterior bladder wall (possible ureter)
D13 Left anterior bladder wall
D14 Right posterior bladder wall
D15 Left posterior bladder wall
D16 Posterior central bladder wall
D17 Posterior central bladder wall
D18 Posterior right bladder wall
D19 Left posterior bladder wall
D20 Posterior central bladder wall
D21 Left anterior edematous area
D22 Posterior central bladder wall
D23 Anterior left bladder wall
D24 Right posterior bladder wall
D25 Right posterior bladder wall
D26 Left posterior bladder wall
D27 Right posterior bladder wall
D28 Left anterior and posterior bladder wall
D29 Deepest right posterior portion of lesion
D30 Left posterior bladder wall
D31 Representative section of dome
D32 Representative section, dome
D33 Representative section of dome
D34 Possible lymph node
D35 Possible lymph nodes

E-Received in formalin, labeled "left pelvic lymph nodes", are multiple pieces of tan-yellow fibroadipose tissue, aggregating to 9.5 x 5.5 x 2.5 cm in size. Twelve possible lymph nodes, ranging in size from 0.5 x 0.3 x 0.2 cm to 3.0 x 2.0 x 1.7 cm in size are dissected. The cut surfaces of all the above lymph nodes are tan-brown with interspersed fatty areas. No discrete lesion is identified, grossly.

The lymph nodes are submitted as follows:

E1-E5 single lymph node, serially sectioned, submitted entirely
E6-E8 single lymph node, serially sectioned, submitted entirely
E9 two lymph nodes, submitted entirely
E10 two possible lymph nodes, submitted entirely
E11 two possible lymph nodes, submitted entirely
E12 four possible lymph nodes, submitted entirely

F-Received in formalin, labeled "right pelvic lymph nodes", are multiple pieces of fibroadipose tissue, aggregating to 8.5 x 6.0 x 2.5 cm in size. Eleven possible lymph nodes, ranging in diameter from 0.5 x 0.5 x 0.2 cm to 3.0 x 2.4 x 2.0 cm in size are dissected. The cut surface of the lymph nodes is tan-pink with interspersed fatty areas.

The lymph nodes are submitted as follows:

F1-F3 single lymph node, serially sectioned, submitted entirely

F4-F5 single lymph node, serially sectioned, submitted entirely
F6-F8 single lymph nod, serially sectioned, submitted entirely
F9 two lymph nodes, submitted entirely

F10 four possible lymph nodes, submitted entirely
F11 single lymph node, submitted entirely

G-Received in formalin labeled as "left common iliac lymph nodes" are multiple fragments of tan-yellow lobulated fibroadipose tissue (5.5 x 4.0 x 0.9 cm). The specimen is predominantly yellow lobulated adipose tissue with streaks of tan fibrosis and multiple possible lymph nodes.

The specimen is serially sectioned and multiple possible lymph nodes are identified ranging in size from 0.4 x 0.3 x 0.2 cm up to 1.1 x 1.0 x 0.5 cm. Multiple lymph nodes are submitted entirely in five cassettes, G1-G5.

Section Code:

G1	one lymph node, bisected
G2	four possible lymph nodes
G3	two possible lymph nodes
G4	two possible lymph nodes
G5	two possible lymph nodes

H-Received in formalin labeled "right common iliac lymph nodes" is a specimen of fibrofatty tissue (5.4 x 2.4 x 1.0 cm). The external surface of the tissue is primarily yellow lobulated adipose tissue. The specimen is serially sectioned and multiple possible lymph nodes are identified ranging in size from (0.5 x 0.3 x 0.3 cm up to 2.0 x 0.8 x 0.6 cm). Representative sections are submitted in four cassettes, H1-H4.

Section Code:

H1	two possible lymph nodes, submitted entirely
H2	one possible lymph node, submitted entirely
H3	two possible lymph nodes, submitted entirely
H4	one lymph node, bisected, submitted entirely

I-Received in formalin labeled, as "pre-sacral lymph nodes" is a 4.0 x 3.0 x 0.5 cm irregular portion of adipose tissue. The specimen is serially sectioned and six lymph nodes are identified ranging in size from 0.4 to 1.3 cm.

Section Code:

I1	one lymph node
I2	two lymph nodes
I3	three lymph nodes

J-Received in formalin labeled, as "para-aortic lymph nodes" are two tan-pink lymph nodes measuring 0.8 and 1.0 cm in greatest dimension each.

Section Code:

J1	one lymph node, trisected
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J2 one lymph node, trisected.

K-Received in formalin labeled, as "pre-caval lymph nodes" are two lymph nodes measuring 2.0 and 3.0 cm.

Section Code:

K1 one lymph node
K2 one lymph node

L-Received in formalin, labeled "discarded segment of bowel", is a segment of bowel (4.4 x 2.5 x 1.6 cm). The specimen is not oriented. The specimen is stapled closed and has a tan-pink serosal surface with attached mesenteric adipose tissue. The specimen is opened and the mucosa is green-brown with normal mucosal folds. There are no polyps, ulcers, or grossly identifiable lesions. Representative sections are submitted in three cassettes, labeled L1-L3.

M-Received in formalin, labeled "proximal margin, left ureter", is a ureter with attached tissue (2.3 x 2.0 x 0.6 cm). The external surface of the tissue is yellow-pink, soft, and lobulated. One staple is attached to the specimen. A ureter is identified and serially sectioned. Cut surface is without lesions or masses. Representative sections are submitted in two cassettes, labeled M1-M2.

Section Code:

M1-M2 proximal margin of left ureter, submitted entirely on end

N-Received in formalin, labeled "distal right ureter", is a ureter specimen with attached tissue (2.3 x 1.1 x 0.5 cm). The external surface of the tissue is tan-pink with attached lobulated adipose tissue. One staple is identified on the specimen. The specimen is serially sectioned and no masses or lesions are identified grossly. The specimen is submitted entirely in cassettes N1-N2.

Section Code:

N1-N2 distal right ureter, serially sectioned, submitted entirely

MICROSCOPIC:

A - Sections show cross sections of ureter with low grade dysplasia involving the urothelium. Obvious high grade dysplasia or invasive carcinoma is not identified.

B - Sections show cross sections of ureter without dysplastic changes.

C - Sections show urothelial together with underlying muscle. There is reactive atypia of the urothelium. Dysplasia is not identified.

D - Sections show urothelial carcinoma, high grade (transitional cell carcinoma grade 3/3). The tumor shows a solid growth pattern. There is minimal papillary formation. The tumor invades into the inner half of the muscularis propria. There are many areas of lymphovascular invasion within the tumor mass, at the tumor advancing edge and away

from the main tumor mass. There is no obvious perineural invasion. The surgical margins (right and left ureteral, urethral and soft tissue margins) are free of tumor involvement.

The urothelium adjacent to the invasive carcinoma show high grade dysplasia/carcinoma in situ.

The urothelium away from the main tumor shows several changes including reactive atypia, low grade dysplasia, high grade dysplasia/carcinoma in situ. Brunner's nests with high grade dysplasia/ carcinoma in situ are present. Several areas of the urothelium are lost. The mucosa shows a few granulomas with multinucleated giant cells. There is also fibrosis, acute and chronic inflammation of the mucosal portion not involved by tumor tissue.

Sections of the right and left ureters show focal areas of high grade dysplasia/carcinoma in situ.

E - Sections show ten lymph nodes. One of them show a microscopic focus of metastatic transitional cell carcinoma.

F - Sections show eleven lymph nodes, two of them show metastatic carcinoma. The maximum size of the metastases are about 3 mm in lymph nodes measuring up to 3.5 mm and 6 mm, respectively.

G - Sections show eleven lymph nodes without metastatic carcinoma.

H - Sections show six lymph nodes without metastatic carcinoma.

I - Sections show six lymph nodes without metastatic carcinoma.

J - Sections show two lymph nodes without metastatic carcinoma.

K - Sections show one lymph node without metastatic carcinoma

L - Sections show terminal ileal wall with no diagnostics alteration.

M - Sections show cross sections of ureter with no dysplasia or carcinoma invasive.

N - Sections show cross section of ureter with focal low grade dysplasia. High grade dysplasia or carcinoma is not identified.

TUMOR SYNOPSIS REPORT AND STAGING:

Specimen: [REDACTED] lymph nodes
Procedure: Radical cystectomy with nodal dissection
Tumor size: 7 cm
[REDACTED]

Histologic type: Urothelial carcinoma, high grade (transition cell carcinoma grade 3/3)

Associated epithelial Lesion: Multifocal high grade dysplasia/ carcinoma in situ

Histologic grade: High grade

Microscopic tumor extension: Involvement of the inner half of the muscularis

Margin status: Negative

Lymphovascular invasion

Multifocal lymphovascular invasion

Primary tumor: T2a

Lymph node

N2

Distant Metastasis: M0

Stage grouping: Stage IV

Site of distant metastases None

[REDACTED]
[REDACTED]
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SURGICAL PATHOLOGY

Dictated by:

This report was verified electronically.

DATE OF DISCHARGE:

DOCTOR:

DATE OF SERVICE:

Pathology Consultation Report

Supplemental

CASE:

REASON FOR ADDENDUM:

Addendum issued to report findings of gross and microscopic examination of prostate following formalin fixation and whole-mount processing.

ANATOMIC PATHOLOGY DIAGNOSIS:

Prostate, radical cystoprostatectomy

- Focal invasive urothelial carcinoma into prostatic stroma
- Extensive urothelial carcinoma in-situ involving prostate, with prostatic duct and acinar involvement
- Urothelial carcinoma in situ involving ejaculatory duct and seminal vesicle
- No lymphovascular invasion seen
- No prostate adenocarcinoma
- Margins negative for urothelial carcinoma

PATHOLOGIST:

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MATERIAL:

O. Prostate.

HISTORY:

with bladder cancer.

GROSS:

O. Received in formalin and labeled prostate is a prostate with attached bilateral seminal vesicles which together weigh 84g. The prostate itself weighs 64g, and measures 4.5cm inferior to superior; 6.0cm transversely; and 6.0cm anterior to posterior.

[REDACTED]

Following formalin fixation, the prostate margins are inked: the right side in black, the left side in blue. Cut surfaces of nonsurgical margins are inked as orange.

The apical margin is removed and submitted in a cone-like fashion. The remainder of the gland is sectioned bilaterally in the sagittal plane along either side of the prostatic urethra. Cut surfaces are smooth pink to tan glistening, with areas of large nodularity and cystic structures. The apex orifice is irregular and dilated to 1.0cm in greatest dimension. The prostatic urethra is hemorrhagic, with focal areas of necrosis.

The seminal vesicles have gray-white, semitranslucent walls of normal thickness.

Section code:

- P1-4 Apex margin
- P5 Whole-mount prostate, sagittal plane, left side
- P6 Whole-mount prostate, sagittal plane, right side
- P7-13 Sequential sections of prostate, apex to base, left side
- P14-21 Sequential sections prostate, apex to base, right side
- P22 Representative sections of left seminal vesicles
- P23 Representative section, right seminal vesicles

MICROSCOPIC:

Sections of prostate show extensive urothelial carcinoma in-situ involving almost entire prostate. There are ductal as well as acinar involvement. Focal stromal invasive carcinoma is seen. In addition, CIS involving ejaculatory duct and seminal vesicle is also noted. No lymphovascular invasion is identified. There is no prostate adenocarcinoma. All margins are negative for tumor.

COMMENT:

Intradepartmental consultation:

(P5 and P6)

TUMOR SYNOPTIC REPORT AND STAGING:

Specimen (include all): Urinary bladder, prostate, extended pelvic lymph node dissection specimens

Procedure type: Radical cystoprostatectomy and extended PLND

Tumor size: 7 cm

Histologic type: Invasive urothelial carcinoma

Associated epithelial lesions: CIS

Histologic grade (low or high): High grade

Microscopic tumor extension: Tumor invades bladder muscularis propria and prostate stroma

Margin status: Negative

Lymphovascular invasion: Yes

Primary tumor (pT): pT4a

Lymph node (pN): pN2

Distant metastasis (M): cM0

Stage grouping: IV

Sites of distant metastasis: Unknown

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SURGICAL PATHOLOGY

Dictated by:

This report was verified electronically.